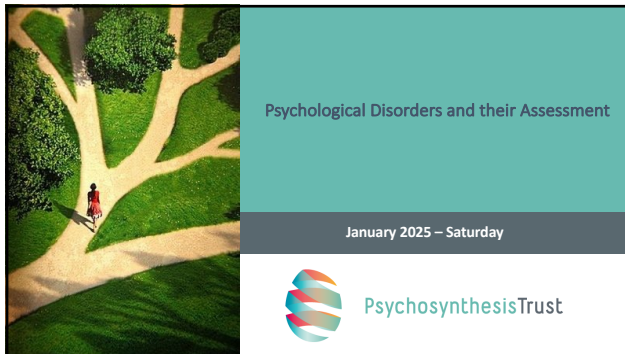
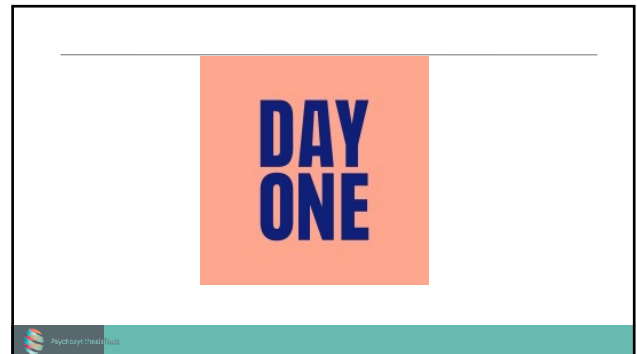




1



2

WHAT WE WILL COVER – SATURDAY 1/2

- Assess client suitability for psychosynthesis counselling.
- undertake a competent initial assessment
- recognising factors that would contra-indicate client suitability.
- Understand the difference between on-line assessment and person to person assessment.
- Understand the Power – Threat – Meaning Framework.

3

SATURDAY – DAY ONE. - A.M.

10:00 am - : Opening and Introduction
 10:30 am – Check in around Psychological Disorders
 11:00 am – break (20 minutes)
 11:30 - - Assessing Client Suitability and Intersectionality

- Understanding the meaning and purpose of assessing client suitability for psychosynthesis counseling
- Noon - Defense mechanisms
- Exploring the difference between online assessment and person-to-person assessment.
- Increasing reflective awareness with a focus on the impact of intersectionality on psychological assessment.

1:00 pm - 2:00 pm: Lunch Break

4

SATURDAY – DAY ONE – P.M.

2:00 pm Practice Section 1

Topic: Initial Assessment and Contra-indications

Developing the ability to undertake a competent initial assessment
Recognizing factors that would contraindicate client suitability.

(Exercise – undertake an assessment – pairs work 30mins each / 1hr)

3:30 pm - 3:50 pm: Midday Break

4:20 pm - 5:00 pm: Wrap-up and Q&A

Recap the key points covered during the day, address any participant questions or concerns.



5

SUNDAY – A.M.

10:00 am - 11:30 am: Lecture Section 2

Topic: Primitive Defense Mechanisms and Mild to Moderate Psychological Disturbances (1.5 hours)

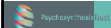
Topic: Assessing for Suicide

- Identifying and describing four primitive defense mechanisms and their purpose, using examples from personal life experience.
- Understanding mild to moderate psychological disturbances.
- 11:00 am – break (20 minutes)

11:50 am - 1:00 pm: Practice Section 2

Topic: Transference, Countertransference, and Intrapersonal/Interpersonal Dynamics (1 hour, 10 minutes)

- Exploring the use of transference and countertransference in assessing intrapersonal and interpersonal dynamics.



6

SUNDAY – P.M.

2:00 pm - 3:30 pm: Lecture Section 3

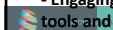
- Topic: Personality Disorders: Narcissistic Personality Disorder, Borderline Personality Disorder (1.5 hours)

- Describing the terms and listing at least two criteria for personality disorders, specifically focusing on narcissistic personality disorder and borderline personality disorder.

3:30 pm - 3:50 pm: Midday Break

3:50 pm - 5:00 pm: Practice Section 3

- Topic: Assessment Tools and Report Writing (1 hour, 10 minutes)
- Engaging participants in exercises to familiarize them with assessment tools and report writing related to the covered topics.

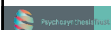


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TWO QUESTIONS:

1. 'What is the purpose of conducting an initial clinical assessment prior to beginning counselling sessions?'
2. What should an initial assessment include?

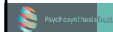
Time: 15 minutes in pairs. Write down your answers.



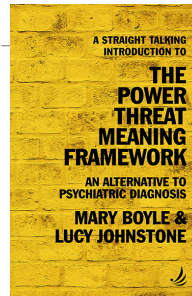
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ASSESSING THE INDIVIDUAL, OURSELVES AND SOCIETY

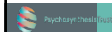
- "How a particular society defines deviance, which may be couched in terms of mental illness, depends on how wide a range of behaviour that society allows to be acceptable... How a society explains deviance from the so-called norm will, in turn, depend on its own prevailing beliefs." Alessandra Lemma (2009)
- Reflective awareness involves how our own cultural, personal and theoretical constructs and assumptions can affect assessment. We need to consider how the intersectional aspects of our lived experience interface with the client's intersectionality and affect our capacity to form a therapeutic alliance. Therefore, reflective awareness in counselling assessment means we need to apply self-assessment.



9



- Many forms of troubling behaviour are understandable when viewed in the context of a person's life.
- Also, the cultural and social norms we are expected to live up to and the degree to which we are exposed to trauma, abuse, injustice and inequality.



10

THE POWER – THREAT – MEANING FRAMEWORK 1/2

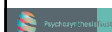
- **Core Principles:** Focus on power dynamics, threats, and personal meaning instead of psychiatric diagnoses.
- **Key Questions:**
 "What happened?" (Power),
 "How did it affect you?" (Threat),
 "What sense did you make of it?" (Meaning),
 "How did you survive?" (Response).



11

THE POWER – THREAT – MEANING FRAMEWORK 1/2

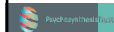
- **Applications:**
 Create hopeful narratives;
 link distress to social factors like poverty and trauma;
 diverse coping strategies.
- **Implications:**
 Influences service design,
 training, and social policy;
 emphasizes cultural sensitivity and non-medical support.



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QUESTION RE: CLIENT SUITABILITY FOR THERAPY

- What factors would make for a client suitable for therapy?
- What Exclusion criteria might exist contraindicating therapy?



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THE POWER THREAT MEANING FRAMEWORK

- What factors would make for a client suitable for therapy?
- What Exclusion criteria might exist contraindicating therapy?



14

Psychological Mindedness

- Client **relates** to their story
- Increasing **awareness**
- Ability to **disidentify**
- Capacity to **symbolise**
- Ability to **tolerate** anxiety, frustration, pain, stress
- Increased **responsibility**
- At least one **"good enough"** relationship
- Capacity to build a **therapeutic alliance**
- **Relief** when given feedback
- Acceptance of the **unconscious**
- Capacity to **self-regulate**

15

Case Study: Chen's Struggle with Adjustment and Academic Performance

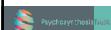
QUESTIONS TO ANSWER

WHAT QUESTIONS MIGHT YOU APPLY FROM BASED ON THE PTM FRAMEWORK?

WHAT INTERSECTIONALITY ISSUES MIGHT YOU NEED TO BE AWARE OF?

IS CHEN SUITABLE FOR THERAPY?

30 MINUTES - GROUPS



16



**BREAK
(20 MINUTES)**

17

POSSIBLE PTM QUESTIONS

1. WHAT HAS HAPPENED TO YOU? (POWER)
 - HOW HAVE THE CHANGES IN YOUR LIFE SINCE MOVING TO LONDON IMPACTED YOU?
 - IN WHAT WAYS DO YOU FEEL THE EXPECTATIONS OF YOUR FAMILY HAVE INFLUENCED YOUR CURRENT SITUATION?
2. HOW DID IT AFFECT YOU? (THREAT)
 - WHAT KINDS OF PRESSURES OR THREATS DO YOU FEEL AS A RESULT OF YOUR ACADEMIC CHALLENGES?
 - HOW HAVE THESE PRESSURES IMPACTED YOUR DAILY LIFE AND WELL-BEING?
3. WHAT SENSE DID YOU MAKE OF IT? (MEANING)
 - HOW DO YOU INTERPRET YOUR ACADEMIC STRUGGLES AND THE IDEA OF SUSPENDING YOUR STUDIES?
 - WHAT MESSAGES HAVE YOU INTERNALIZED ABOUT YOUR SELF-WORTH AND ABILITIES?
4. WHAT DID YOU HAVE TO DO TO SURVIVE? (THREAT RESPONSE)
 - WHAT STRATEGIES HAVE YOU USED TO COPE WITH THE STRESS AND OVERWHELMING FEELINGS SINCE ARRIVING IN LONDON?
 - HOW HAVE THESE STRATEGIES HELPED OR HINDERED YOUR ABILITY TO MANAGE YOUR ACADEMIC RESPONSIBILITIES?

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POSSIBLE PTM RESPONSES

5. WHAT ARE YOUR STRENGTHS? (POWER RESOURCES)
 - WHAT SKILLS OR RESOURCES DO YOU FEEL YOU HAVE THAT COULD HELP YOU NAVIGATE THESE CHALLENGES?
 - WHO IN YOUR LIFE, EITHER HERE OR BACK HOME, COULD OFFER YOU SUPPORT?
6. WHAT IS YOUR STORY? (PERSONAL NARRATIVE)
 - HOW WOULD YOU DESCRIBE YOUR JOURNEY SO FAR, INCLUDING THE CHALLENGES AND SUCCESSSES?
 - WHAT DO YOU WANT YOUR NARRATIVE TO BE MOVING FORWARD, AND HOW CAN WE SUPPORT YOU IN SHAPING THAT STORY?

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Client suitability for therapy

- “Good enough” level of **insight**
- **Previous positive experience** of therapy / counselling
- Current **attitude** to therapy
- Client’s **curiosity** about their inner world
- Willingness to take **responsibility** for own actions / behaviours
- How has client been **referred** for therapy?
- Client has had the experience of **at least one “good enough” relationship**
- **Support network**: availability of, and client’s willingness / ability to use it
- **Psychological mindedness** vs. psychological sophistication

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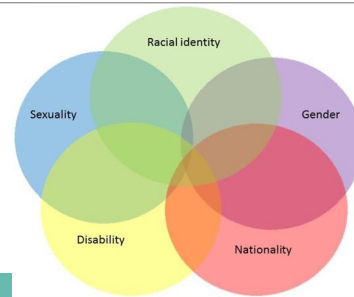
Exclusion Factors – why the client may not be suitable for therapy

The client:

- has been diagnosed with a **personality disorder**
- has had previous therapy with **no** improvement or perceived benefit
- presents with severe **chemical addiction**
- has made **suicide attempts recently**
- demonstrates **severe acting out and self-destructive behaviour**
- has received recent and/or repeated **ECT treatment**
- has been recently **hospitalised** for a psychiatric disorder
- is on **psychotropic** or other medication (depending on medication)
- presents with **severe borderline** (emotional instability disorder) or **severe narcissistic traits**
- presents with **psychosis**
- In or awaiting **trial or court proceedings**
- Some severe eating disorders eg **anorexia nervosa**

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INTERSECTIONALITY



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DEFINITION OF INTERSECTIONALITY

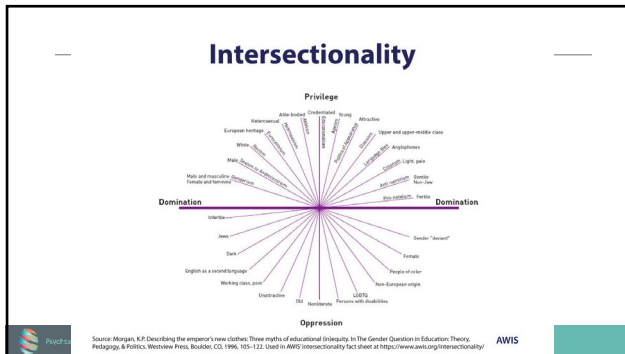
- , the Oxford Dictionary defines intersectionality as “the interconnected nature of social categorisations such as race, class, and gender, regarded as creating overlapping and interdependent systems of discrimination or disadvantage”.

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QUESTION: RE: INTERSECTIONALITY

- **What polar opposites of intersectionality might exist? (e.g., how would you design a graphic model of intersectionality)**
- **E.g., in terms of Privileged vs. Oppressed?**

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25



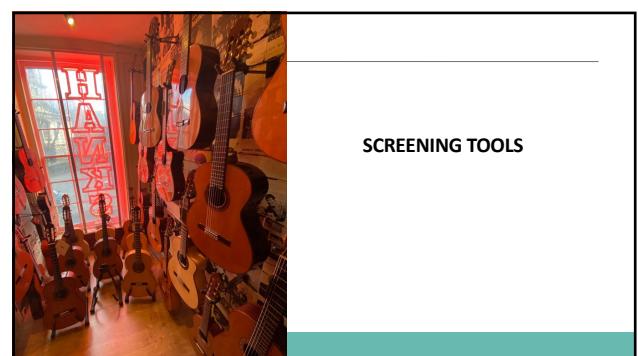
26

EXERCISE: CLIENT SUITABILITY 'RAHUL'

- Suitability Assessment:**
 Based on the information provided, evaluate Rahul's suitability for psychosynthesis therapy.
 Consider his presenting concerns, background, current mental health state, and motivations for seeking help.
 Explain your reasoning, highlighting any factors that might support or hinder effective engagement with psychosynthesis principles.
- Decision and Rationale:**
Would you choose to work with Rahul at this time?
 Justify your decision, considering both his potential needs and your capacity to support them within the framework of psychosynthesis.

TIME: 40 minutes - Break up into three groups and decide

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INITIAL SCREENING TOOLS

PHQ-9, GAD-7, and CORE-10

- PHQ-9, GAD-7, and CORE-10 are all brief screening tools used to assess for symptoms of mental health conditions. It's important to remember that

They are not diagnostic tools and cannot definitively diagnose any disorder.

However, they can be helpful indicators that may prompt further evaluation by a qualified mental health professional. Here's a breakdown of each:

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GAD-7, PHQ-9 and CORE-10

ACRONYMS

- GAD-7 = **Generalized Anxiety Disorder 7-item.**
- PHQ-9 = **Patient Health Questionnaire-9.**
- CORE-10 = **Clinical Outcomes in Routine Evaluation**

30

Clinical Outcomes in Routine Evaluation 10 (CORE-10)

The core-10 forms are a set of basic emotions or feelings that are commonly experienced by individuals.

As a trainee on a counseling course, understanding and working with these forms can be helpful in your therapeutic practice.

31

What do Core-10 scores mean?

- Scores on the CORE-10 can be divided into distress categories:
- Healthy (0–5)**
- low (6–10)**
- mild (11–14)**
- moderate (15–19)**
- moderate-to-severe (20–24)**
- and severe (25 and above).**

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GAD-7 and PHQ-9

PHQ-9

- **PHQ-9:**
- **Purpose:** A 9-question screening tool for depression symptoms.
- **Measures:** Frequency and severity of depressive symptoms like low mood, fatigue, and changes in appetite/sleep over the past two weeks.

GAD-7:

- **Purpose:** A 7-question screening tool for generalized anxiety disorder (GAD) symptoms.
- **Measures:** Frequency and severity of GAD symptoms like excessive worry, restlessness, and feeling on edge over the past two weeks.

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GAD-7 AND PHQ-9

• GAD-7. Measures Anxiety

The **Generalized Anxiety Disorder scale** (GAD-7) is one of the most frequently used diagnostic self-report scales for screening, diagnosis and severity assessment of anxiety disorder.

- GAD-7 Scores represent: **0-5 mild; 6-10 moderate; 11-15 moderately severe anxiety 15-21 severe anxiety.**

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PHQ-9

- **Patient Health Questionnaire (PHQ-9). Measures Depression**
PHQ-9 is self-administered.
It takes less than 3 minutes to complete and simply scores each of the 9 [DSM-IV](#) criteria for depression
- **PHQ SCORES:** 0-5 = mild 6-10 = moderate 11-15 = moderately severe 16-20 = severe depression

35



36

ONLINE VS IN-PERSON THERAPY



37

ONLINE VS IN-PERSON THERAPY

- What are key differences between online vs person-to-person assessments?
- What are advantages and disadvantages of each?

(pairs 15 minutes)

38

online assessment for counselling vs person-to-person assessments (1/4)

- 1. Lack of physical presence:
Online assessments for counselling lack the physical presence of the counsellor, which can make it difficult to establish a personal connection and build trust.
- 2. Limited non-verbal communication:
Online assessments rely heavily on verbal communication, which can limit the ability to pick up on non-verbal cues such as body language and facial expressions.

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online assessment for counselling vs person-to-person assessments (2/4)

- 3. Technical issues:
Online assessments can be affected by technical issues such as poor internet connection, which can disrupt the flow of the session and impact the quality of the assessment.
- 4. Limited scope of assessment:
Online assessments may not be able to provide a comprehensive assessment of the client's mental health, as they may not have access to all the necessary information or be able to observe the client in their natural environment.

40

online assessment for counselling vs person-to-person assessments (3/4)

- 5. Privacy concerns:
Online assessments may raise privacy concerns, as the client may not feel comfortable discussing sensitive issues in an online setting, or may be concerned about the security of their personal information.
- 6. Online assessments may require different therapeutic approaches: ic approaches than person-to-person assessments, as the counsellor may need to adapt their techniques to the online environment and the limitations it presents.

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online assessment for counselling vs person-to-person assessments (4/4)

- 7. Limited physical interventions:
Online assessments may not allow for physical interventions such as touch or movement-based therapies, which can be important in certain types of counselling.
- 8. Different levels of engagement:
Online assessments may require different levels of engagement from the client, as they may be more distracted or less invested in the session due to the lack of physical presence.

42

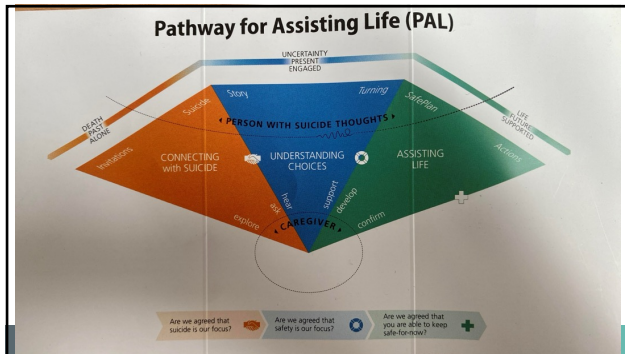
Suicide Prevention



43



44



45

SUICIDE/Self-Harm Video (14:43)

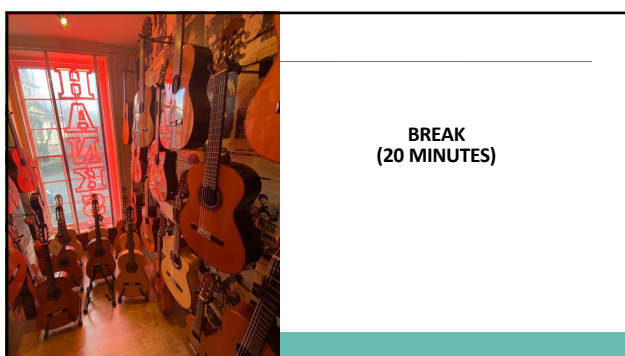
Self-Harm & Suicide

ASIST Suicide Intervention Model

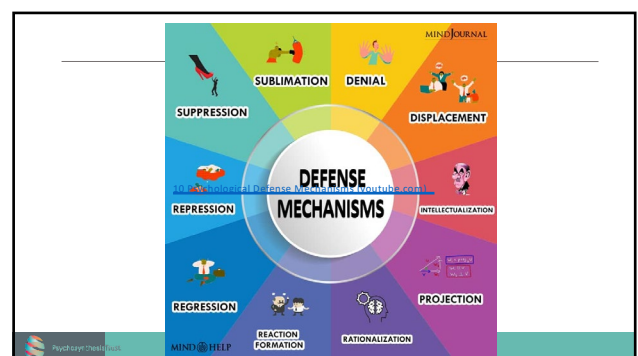
The Pathway for Assisting Life (PAL) Model

1. Connecting w/ Suicide (I care!)
 - a. Explore stressful events w/ feelings of loss & other risk factors.
 - b. Ask the question: "What led to this point?"
2. Understanding Choices (I understand!)
 - a. Hear their story, get the background.
 - b. Support their turning point has to be their choice.
3. Assisting Life (I'll help!)
 - a. Develop a safety plan "What will help keep you safe for now?"
 - b. Confirm actions identify the most important part of the safety plan and the first steps needed to achieve it.

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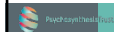


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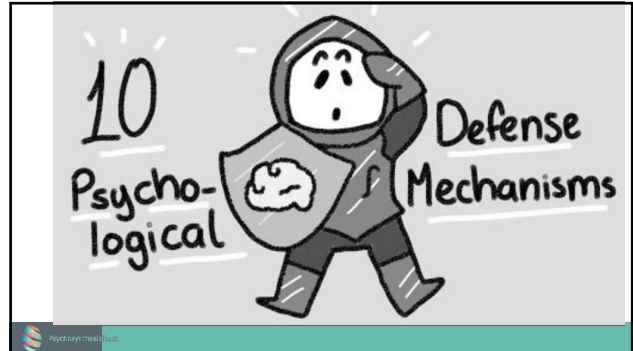
DEFENSE MECHANISMS

QUESTIONS:

1. What are defense mechanisms and what are they for?
2. Do they happen consciously or unconsciously?



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DEFENSE MECHANISMS

- Definition:
Defense mechanisms are unconscious behaviours or psychological strategies people use to avoid experiencing anxiety, discomfort, or threats to the ego.
- People are typically not aware when they're using these self-protective methods.
- The concept of defense mechanisms stems from Sigmund Freud's psychoanalytic theory and was developed in detail by his daughter Anna Freud, who in 1937 published her book [The Ego and the Mechanisms of Defence](#) describing 10 main defense mechanisms.



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"A defence mechanism is a manipulation of perception intended to protect the person from anxiety."
(Kahn, 1997)

Some Defence Mechanisms:

Denial
Splitting
Projective Identification
Idealisation
Projection
Dissociation
Repression
Intellectualisation
Minimising
Sublimation
Humour
Spiritual bypassing



52

SUNDAY – A.M.

10:00 am - 11:30 am:

Topic: Transference, Countertransference,

- Exploring the use of transference and countertransference in assessing intrapersonal and interpersonal dynamics.

11:00 am – break (20 minutes)

11:20 – 11:50: EXERCISE: Psychoeducation about unhelpful thought patterns as an assessment tool

11:50 am - 1:00 pm - Diagnostics / GA-7 PHQ-9 / Topic: Session note – a quick word about therapy notes / report

1:00 pm - 2:00 pm: Lunch Break

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SUNDAY – P.M.

2:00 pm - 3:30 pm: -

Topic: Mild to Moderate Psychological Disturbances

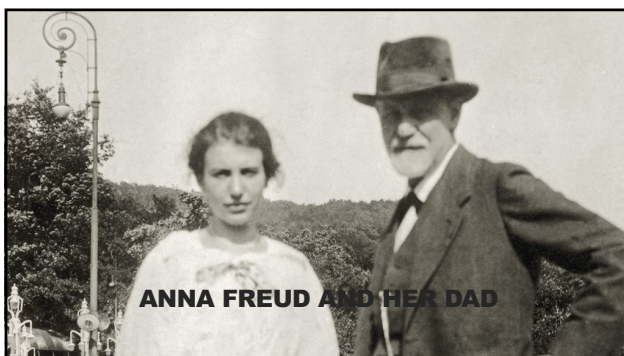
Topic: Personality Disorders:

-

3:30 pm - 3:50 pm: Midday Break

3:50 pm - 5:00 pm: Topic: Case Study Exercise: BPD

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ANNA FREUD AND HER DAD

55

ANNA FREUD'S 10 DEFENSE MECHANISMS 1- 5

- Repression: The ego pushes unacceptable thoughts, feelings, or memories into the unconscious.
- Regression: The ego reverts to a more infantile stage of development in response to stress.
- Reaction formation: The ego adopts an extreme attitude that is the opposite of the true feeling.
- Projection: The ego attributes its own unacceptable thoughts, feelings, or motives to someone else.
- Rationalization: The ego provides a logical explanation for behavior that is actually motivated by unconscious factors.

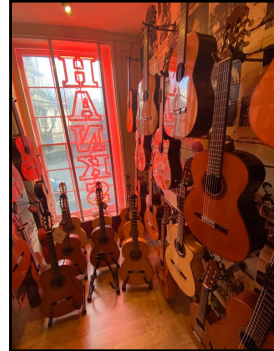
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ANNA FREUD'S 10 DEFENSE MECHANISMS 6-10

- **Isolation:** The ego separates the emotional component from a traumatic event, allowing the person to think about the event without feeling any emotion.
- **Sublimation:** The ego redirects unacceptable impulses into socially acceptable activities.
- **Undoing:** The ego performs an action that is the opposite of an unacceptable impulse, in an attempt to cancel out the impulse.
- **Somatization:** The ego converts anxiety into physical symptoms.



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**BREAK
(20 MINUTES)**

58

EXERCISE: RE: MENTAL STATUS EXAM CONSIDERATIONS

Question:

List at least five considerations during an assessment of mental health

Pairs: 10 minutes



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NINE MENTAL STATUS EXAM CONSIDERATIONS

1. Appearance
2. Behaviour / Psychomotor Activity
3. Attitude toward the Counsellor
4. Mood (affect)
5. Speech and thought
6. Perceptual disturbances
7. Orientation and consciousness
8. Memory / intelligence
9. Reliability, judgement and insight



60

"Our whole spiritual transformation brings us to the point where we realise that in our own being, we are enough"

Ram Dass

"We set ourselves up as wise and trustworthy, armed with skills that will help people to unravel their problems. Yet, we know we are still anxious, still prone to fright, uncertainty and depression, aware of how unpractised we are and how little we know."

Nina Coltart
Art of Assessment

61

WHAT WE WILL COVER – SATURDAY 2/2



- Understanding the difference between on-line assessment and person to person assessment.
- Increase reflective awareness with a focus on the impact of intersectionality and psychological assessment.
- Taking risks in therapy. Even at the initial assessment.

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WHAT WE WILL COVER – SUNDAY

- Defence Mechanisms / Transference Counter-transference, *i.e., understanding of the use of transference and counter transference in assessing intra-personal and interpersonal dynamics.*
- Risk assessment
- Session note writing
- An understanding of mild to moderate psychological disturbances.
- Personality disorders - narcissistic personality disorder, borderline personality disorder



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client assessment essentials

- **Overview:**
 - i) Is the client able to use what I offer?
 - client's internal structure
 - client's external relationships and attachment
 - ii) - Can I offer what the client needs or is looking for?
 - experience and resources
 - iii) - Holding bifocal vision; what is trying to emerge?
- **What does the assessment include?**
 - Why are they seeking counselling now? What is their motivation?
 - Psychological mindedness
 - Inclusion factors
 - Exclusion factors
 - Risk assessment (BACP Ethical Framework, 2018)
 - Biographical information

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VIDEO: IRVIN YALOM – TAKING RISKS

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EXERCISE: TAKING RISKS

Task: In pairs, simulate an initial assessment.

You have taken the essential information and you still have 20 minutes left to elicit an unconscious desire of the client.

The desire is about what they want and have yet not expressed. Elicit this in some creative way, perhaps asking about a dream or some other method.

TIME: in pairs, 20 minutes each way.

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DAY TWO TOPICS

- Transference – counter-transference (*continued*)
- Risk Assessment
- Session note writing
- Exploring Negative Automatic Thoughts as an assessment tool
- An understanding of mild to moderate psychological disturbances.

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Hypothesising about the Assessment

Need three ways of thinking about task of assessment

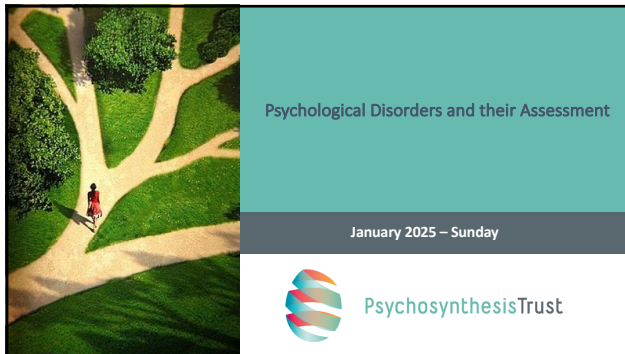
- Assess what is going on within the client (*intrapersonal dynamics*)
- Assess what's going on between you (*interpersonal dynamics*)
- Assess what might be *emerging* for the client

Formulate working hypothesis and strategy

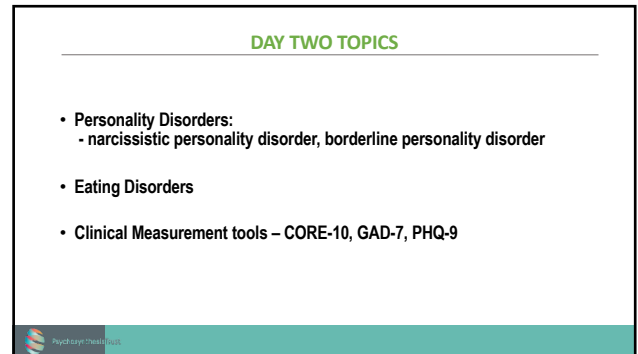
Carefully consider:

- a) Client *motivation*
- b) How the client responds :their ability to *reflect*
- c) *Rigidity* of defence structure
- d) *Risks*
- e) *Counter transference*

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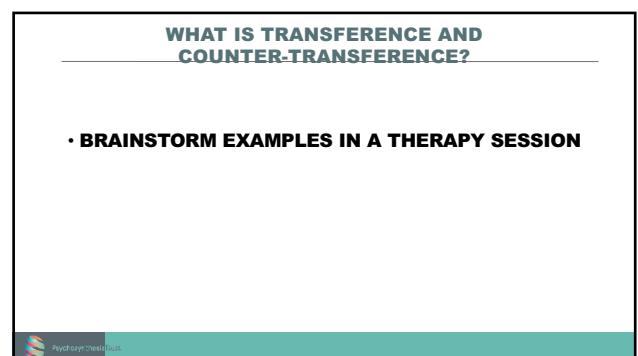
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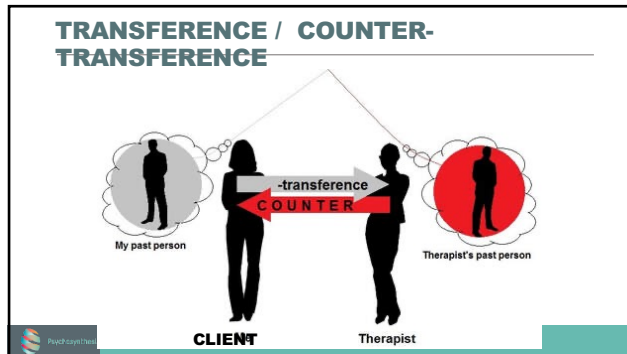
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TRANSFERENCE

- **Client's perspective:** It's the unconscious redirection of feelings and attitudes from significant figures in a client's past (like parents or caregivers) onto the therapist.
- **What it looks like:** The client might feel intense positive emotions like idealisation or see the therapist as a source of strength, or they might experience negativity like anger or distrust based on past experiences.
- **Role of the therapist:** The therapist helps the client understand and process these feelings in a healthy way, using them to gain insight into their past relationships and develop new patterns of interaction.

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COUNTER-TRANSFERENCE

- **Therapist's perspective:** It's the therapist's own unconscious feelings and experiences triggered by the client.
- **What it looks like:** The therapist might feel empathy, warmth, or even frustration or irritation towards the client.
- **Importance of management:** While some emotions like empathy can be helpful for building rapport, strong countertransference reactions need to be managed through self-awareness and supervision to avoid clouding judgment or impacting the therapeutic relationship negatively.

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Transference & Counter Transference – the Transferential Field

- **Transference** – '...transference is defined as the conscious and unconscious responses –both affective and cognitive- of the patient to the therapist' (Maroda, 1991:66)
- 'transference is the expression of the many different layers of the client's personality as they are surfaced by an intimate empathic relationship' (Firman & Gila, 1997:237)
- **Counter transference** – the counter transference is defined as the conscious and unconscious responses of the therapist to the patient (Maroda, 1991:66)
- 'All feelings and attitudes about the client that occur in a therapist' (Kahn, 1991:118)
- **Transferential field** – what is in the field, e.g. visual images, body sensations/language, fantasies. A constant interaction between client and therapist that is mostly unconscious and which may or may not have a causal relationship

76

WHAT IS TRANSFERENCE AND COUNTER-TRANSFERENCE?

• The 'unthought known' – a not knowing yet experiencing state:

i) During the session the trainee counsellor is responding and **listening to** the client but on listening back to the recording the student **hears** the client being a 'good girl/boy to their 'good mother/father'.

ii) During the session the trainee feels ungrounded, light-headed and distracted and then the client reports that they can't remember something. There is some dissociation in the **transferential field**.

Both client and student are unable to tolerate something and dissociate.

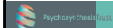
iii) During the session the trainee **realises** they are both holding their breath. The trainee asks the client to breathe deeply and both client and student start to relax.



77

WHAT IS TRANSFERENCE AND COUNTER-TRANSFERENCE?

- **Mostly unconscious but partly conscious**
- **An instrument of research into the clients unconscious**



78

Risk Assessment

- Is the client well enough to benefit from counselling at this time or do they need a referral to mental health services?
- Do you need to liaise with other health professionals?
- How can risk be reduced?
- BACP – Ethical framework consider requirements to do no harm, integrity, humility, candour
- **Drugs and Alcohol** assessment
- **Suicidal indicators:** ideation/recent attempts
- **Self harm**
- Recent episodes of serious acting out eg use of **violence**

79

QUESTIONS: Risk Assessment

EXERCISE:

Simulate an assessment with a client who has completed a CORE-10 FORM with high-risk indications.

1) WHAT ARE THE MOST FACTORS DO YOU CONSIDER WHEN ASSESSING RISK?

2) WHAT QUESTIONS DO YOU ASK WHEN DOING A RISK ASSESSMENT?

80

What questions do I ask?

- 1) Risk assessment
- 2) Support network
- 3) Previous experience of counselling
- 4) Why now?
- 5) Biographical data – relationships with family of origin as a child and now, siblings, school experience, relationships, work, disabilities and health issues, current living arrangements
- 6) Significant events and /or losses
- 7) What makes their heart sing or sink?

Useful questions...

- What brings you here?
- Why now?
- Tell me about the relationships that support you
- What is going on for you when you talk about this experience?
- How is it to be here with me?
- What was your experience of the counselling you have had in the past? What worked and what didn't work so well?
- Without going into a lot of detail today, can you tell me about any significant events in your past?

81

Suicidality vs Self-harm ^{1/2}

Question:
How do you distinguish between self-harm and suicidal behaviour?

- **Self-harming clients** may have a history of **repeated, non-lethal self-injury**, whereas suicidal clients may have a history of previous suicide attempts or escalating suicidal ideation.

- **Suicidal clients** often **express a desire to end their life**, while self-harming clients typically use self-injury as a coping mechanism without suicidal intent.

82

Contrasts: Self-harm vs Suicidality

- **Self-harm:** Expressing feelings of emotional pain, using self-harm to relieve distress;
- **Suicidal:** Expressing feelings of hopelessness, believing that life is not worth living
- **Self-harm:** Engaging in impulsive or ritualistic self-injury without a lethal plan
- **Suicidal:** Having a specific plan for ending one's life, such as a chosen method or timeframe.
- **Self-harm:** Concealing self-harm, wearing clothing to cover scars or fresh injuries
- **Suicidal:** Giving away possessions, making final arrangements, saying goodbye to loved ones
- **Self-harm:** Seeking help or support, expressing a desire to stop self-harming behaviour
- **Suicidal:** Withdrawing from social support, feeling like a burden to others

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EXERCISE: TRANSFERENCE / COUNTER-TRANSFERENCE

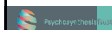
• FIVE GROUPS – FIVE SCENARIOS

• **TIME:** 20 minutes each in pairs.

Total 40 minutes

• Debrief – 5 minutes each

• **TOTAL – 45 MINUTES**



84

The Power of a Single Session:
'One at a Time' Counselling – Univ Bristol

1 At a time
counselling

85

Clues
Investigating
Solutions in
Brief Therapy

Steve de Shazer

86



87

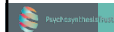
**PSYCHOLOGICAL
DISORDERS**



88

QUESTION:

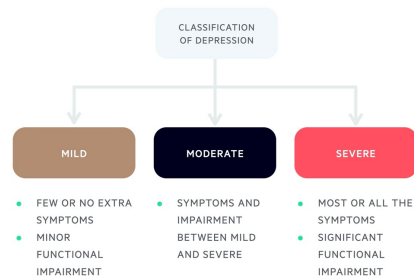
- 'What are your hopes and fears about learnings about topics such as:
- neurosis
- psychosis
- spiritual emergencies
- mental illnesses?'



89



90

Understanding mild to moderate psychological disturbances

91

Psychological Disorders as a Continuum

No Disorder	Mild Disorder	Moderate Disorder	Severe Disorder
Absence of signs of psychological disorder	Few signs of distress or other indicators of psychological disorder	Indicators of disorders are more pronounced and occur more frequently	Clear signs of psychological disorder, which dominate the person's life
Absence of behavioral problems	Few behavior problems; responses usually appropriate to the situation	More distinct behavior is often inappropriate to the situation	Severe and frequent behavior problems; behavior is usually inappropriate to the situation
No problems with interpersonal relationships	Few difficulties with relationships	More frequent difficulties with relationships	Many poor relationships or lack of relationships

Disorders are exaggerations of normal behavior and responses.

92

Differences between Neurosis and Psychosis

Neurosis

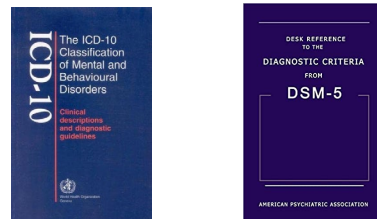
- the individual's attempt to deal with **anxiety**
- is usually connected to a life situation
- the issue is inner conflict
- there is sense of 'I', reflective capacity and ability to make choices

Psychosis

- loss of reality testing and impairment of mental functioning
- is manifested by delusions, hallucinations and impaired memory
- often characterised by severe paranoia

93

ICD 10 – INTERNATIONAL CLASSIFICATION OF DISEASES DSM 5 – DIAGNOSTIC STATISTICAL MANUAL OF MENTAL DISORDERS



94

PERSONALITY DISORDER - DEFINITION

- A personality disorder is a type of mental disorder in involving an unhealthy pattern of thinking, functioning and behaving.
- A person with a personality disorder has trouble perceiving and relating to situations and people.

Three clusters of personality disorders:

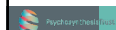
- Suspicious, odd or eccentric disorders;
- dramatic, emotional or erratic disorders;
- anxious or fearful disorders



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PERSONALITY DISORDER DEFINITIONS 1/2

- Borderline personality disorder (BPD)** is a pervasive pattern of instability of interpersonal relationships, self-image, moods, and control over impulses.
- Obsessive-compulsive personality disorder** is a pervasive pattern of preoccupation with orderliness, perfectionism, and mental and interpersonal control, at the expense of flexibility, openness, and efficiency. **Paranoid personality disorder** is a pervasive distrust and suspiciousness of others, such that their motives are interpreted as malevolent.
- Schizoid personality disorder** is a pervasive pattern of detachment from social relationships and a restricted range of expression of emotions in interpersonal settings.
- Schizotypal personality disorder** is a pervasive pattern of social and interpersonal deficits marked by acute discomfort with reduced capacity for close relationships. It is also characterised by distortions of thinking and perception and eccentric behaviour.



96

PERSONALITY DISORDER DEFINITIONS 2/2

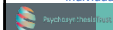
- **Narcissistic personality disorder** is a pervasive pattern of grandiosity (in fantasy or actual behaviour), need for admiration, and lack of empathy.
- **Avoidant personality disorder** is a pervasive pattern of social inhibition, feelings of inadequacy, and hypersensitivity to negative evaluation.
- **Dependent personality disorder** is a pervasive and excessive need to be taken care of, which leads to submissive and clinging behaviour and fears of separation.
- **Histrionic personality disorder** is a pervasive pattern of excessive emotion and attention seeking.
- **Antisocial personality disorder** is a pervasive pattern of disregard for and violation of the rights of others.



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PERSONALITY DISORDERS

- Mental health conditions that affect how people [manage their feelings](#) and how they [relate](#) to other people.
- **DSM-5 Cluster B:** personality disorders characterised by [erratic and dramatic behaviours](#) and [emotional instability](#).
- This cluster includes borderline and narcissistic personality disorders.
- People with personality disorders are often considered difficult to treat, because they [elicit strong emotional reactions from others](#) around them.
- People with personality disorders [may have no awareness or some awareness](#) that their personality traits are compromising their relationships and effectiveness in major aspects of life.
- Paradoxically, many people with personality disorders [may be successful and highly functioning individuals](#) in some areas of life and therefore the diagnosis may be harder to determine.



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TYPES OF PERSONALITY DISORDER - 1

- Emotional and impulsive:
- [Borderline personality disorder \(BPD\)](#)
- [Antisocial personality disorder \(ASPD\)](#)
- [Histrionic personality disorder](#)
- [Narcissistic personality disorder](#)

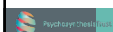


99

TYPES OF PERSONALITY DISORDER - 2

Anxious:

- [Obsessive compulsive personality disorder \(OCPD\)](#)
- [See NICHE Guidelines for OCD and Body Dysmorphia:](#)
- [Avoidant personality disorder](#)
- [Dependent personality disorder](#)

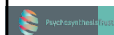


100

TYPES PERSONALITY DISORDER - 3

Suspicious:

- [Paranoid personality disorder](#)
- [Schizoid personality disorder](#)
- [Schizotypal personality disorder](#)



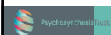
101

SCHIZOID VS SCHIZOTYPAL DIFFERENCES

- Schizoid types do not have any paranoia or suspiciousness.
- This means that people with a schizoid diagnosis will not be worried about other people's motivations or feel that others are out to get them.
- However Schizotypal people will have these paranoid fears.

ISOLATION TENDENCY DIFFERENCES

- For schizotypal types the isolation and lack of close friendships is due to social anxiety or eccentricity.
- For schizoid types the isolation is due to a lack of interest in others people.



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SCHIZOID VS SCHIZOTYPAL DIFFERENCES

Feature	Schizoid Personality Disorder	Schizotypal Personality Disorder
Desire for Social Relationships	Low	May desire them but struggle
Thoughts and Perceptions	Normal	Odd beliefs, magical thinking, paranoia
Emotional Expression	Restricted range	May be normal but sometimes inappropriate
Seeking Help	Unlikely	More likely for associated anxiety or depression



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PARANOID PERSONALITY DISORDER – (PPD)

- PPD is a mental health condition marked by a pattern of distrust and suspicion of others without adequate reason to be suspicious.
- PPD people are always on guard, believing others are constantly trying to demean, harm or threaten them.



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Borderline Personality (DSM 5 criteria) aka Emotionally Unstable Personality Disorder

- Pervasive pattern of instability in interpersonal relationships and self-image and at least 5 of the following:
- Chronic feelings of **emptiness**
- Extreme **emotional instability** in reaction to day to day events
- Frantic efforts to **avoid abandonment**
- **Identity disturbance** with unstable image of self or sense of self
- **Impulsive behaviour** in at least two areas that are potentially self-damaging eg spending, sex, reckless driving, substance abuse
- **Inappropriate anger** or frequent displays of temper
- Extreme **splitting** involving idealization and devaluation in intense personal relationships
- Recurrent **suicidal behaviour**, gestures or threats
- **Self-harming**

Useful books

- Kreisman & Straus (1989) I hate you, don't leave me
- Chapman & Gratz (2007) The Borderline Personality Disorder
- Moskowitz (2001) Lost in the Mirror

Don't Panic!
We all have symptoms. The condition is marked by the severity with which the symptoms present

105

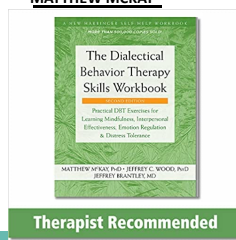
BORDERLINE PERSONALITY SYMPTOMS

- Impulsivity
- Extreme mood swings
- Intense fear of abandonment
- Self-harming behaviour
- Poor anger control
- Suicidal or self-mutilating behaviours
- Feelings of emptiness

106

DIALECTICAL BEHAVIOR THERAPY SKILLS WORKBOOK

MATTHEW MCKAY



Therapist Recommended

107

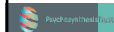
THREE STAGES OF THERAPY APPROACHES W BPD. (1/2)

- Initial Phase
 - Focus on suicidal behaviour.
 - Therapy-interrupting behaviours
 - Behaviour that interfere with the quality of life
 - Offer help to develop the necessary skills to resolve these problems.

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THREE STAGES OF THERAPY APPROACHES W/ BPD. (1/2)

- **Middle Phase**
 - Deal with post-traumatic stress related problems (PTSD)
- **End Phase**
 - Focus on self-esteem and individual treatment goals.



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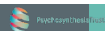
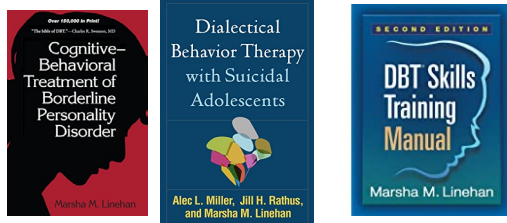
EXERCISE: BORDERLINE PERSONALITY CASE STUDY – ELLEN'S STORY: A PSYCHOSYNTHESIS CASE STUDY

- **TASK – 1)** Devise an assessment. Include a draft 3-stage Treatment Plan and highlight your *potential* interventions in
 - Beginning stage
 - Middle stage
 - closing stage
- **TIME:** 40 Minutes in small groups



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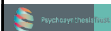
DIALECTICAL BEHAVIOR THERAPY BOOKS MARSHA LINEHAN



111

THERAPY INTERRUPTING BEHAVIOURS - MARSHA LINEHAN

1. **NON-ATTENDANCE OR LATENESS:** CONSISTENTLY MISSING THERAPY SESSIONS OR ARRIVING LATE TO APPOINTMENTS.
2. **NON-COLLABORATION:** REFUSING TO WORK WITH THE THERAPIST, NOT ENGAGING IN THE THERAPEUTIC PROCESS, OR NOT COMPLETING AGREED-UPON TASKS OR HOMEWORK.
3. **NON-COMPLIANCE:** NOT FOLLOWING THROUGH WITH THE THERAPIST'S RECOMMENDATIONS OR NOT APPLYING THE SKILLS LEARNED IN THERAPY TO REAL-LIFE SITUATIONS.
4. **AVOIDANCE:** AVOIDING DISCUSSING PAINFUL OR UNCOMFORTABLE TOPICS, EMOTIONS, OR SITUATIONS DURING THERAPY SESSIONS.
5. **WITHHOLDING INFORMATION:** NOT DISCLOSING RELEVANT INFORMATION OR NOT BEING FULLY HONEST WITH THE THERAPIST ABOUT THOUGHTS, FEELINGS, OR BEHAVIORS.



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THERAPY INTERRUPTING BEHAVIOURS - MARSHA LINEHAN

6. ATTACKING OR THREATENING THE THERAPIST: ENGAGING IN VERBAL OR PHYSICAL AGGRESSION TOWARDS THE THERAPIST OR MAKING THREATS OF HARM.

7. BOUNDARY VIOLATIONS: ATTEMPTING TO EXTEND THE THERAPEUTIC RELATIONSHIP BEYOND PROFESSIONAL BOUNDARIES, SUCH AS MAKING EXCESSIVE PHONE CALLS OR DEMANDING MORE TIME OR ATTENTION FROM THE THERAPIST.

8. SUBSTANCE ABUSE: USING DRUGS OR ALCOHOL IN A WAY THAT INTERFERES WITH THE ABILITY TO ENGAGE IN THERAPY OR APPLY THE SKILLS LEARNED.

9. SELF-HARM OR SUICIDAL BEHAVIORS: ENGAGING IN SELF-INJURIOUS BEHAVIORS OR EXPRESSING SUICIDAL THOUGHTS OR INTENTIONS, WHICH MAY REQUIRE IMMEDIATE INTERVENTION AND DISRUPT THE REGULAR COURSE OF THERAPY.

10. SPLITTING: VIEWING THE THERAPIST IN EXTREME, BLACK-OR-WHITE TERMS, SUCH AS IDEALIZING THEM ONE MOMENT AND DEVALUING THEM THE NEXT, WHICH CAN LEAD TO UNSTABLE THERAPEUTIC

113

Narcissistic Personality Disorder – DSM 5

- Excessive reference to others for self-definition and self esteem regulation (inflated and/or deflated, and vacillates between extremes)
- Self direction- goal setting based on gaining approval from others. Personal standards unreasonably high to be seen as 'exceptional' or too low based on sense of entitlement
- Empathy- Impaired ability to identify or recognise other people's feelings and needs, excessively attuned to reactions of others but only if relevant to self. Over or under estimates own effect on others
- Intimacy – relationships are superficial and only exist to serve self esteem regulation. Little genuine interest in others and predominance of personal gain.
- Antagonism characterised by grandiosity and attention-seeking
- Traits are stable over time and situation
- Traits are not better understood as normative to developmental stage or socio-cultural environment
- Traits are not solely due to drugs, medication or head trauma
- Books
 - Lowen (1985) Narcissism
 - Johnson (1987) Humanising the Narcissistic Style
 - McBride (2008) Will I ever be good enough? Healing the daughters of narcissistic mothers

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Psychosynthesis Trust



Mike Bryant, Psychosynthesis Trust
Trainer and Supervisor
interviews
Andy Bell, Deputy Chief Executive at
Centre for Mental Health

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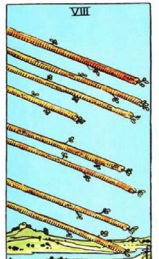


Psychosynthesis Trust

SUPPLEMENTARY INFO

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SESSION NOTES WRITING



Psychology Training

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WRITING SESSION NOTES(1/2)

- 1.Be comprehensive.**
Therapy notes should be comprehensive and should include all of the relevant information about the client's session.
This may include the client's presenting problem, their goals for therapy, the interventions that were used, and the client's progress.
- 2.Be objective.**
Therapy notes should be objective and should not include the counsellor's opinions or interpretations.
The notes should simply record what the client said and did, and how the counsellor responded.

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WRITING SESSION NOTES

(2/2)

- 3. Be confidential.**
Therapy notes are confidential and should not be shared with anyone without the client's consent.
- 4. Be accurate.**
Therapy notes should be accurate and should reflect the actual events of the session.
- 5. Be timely.**
Therapy notes should be written as soon as possible after the session, while the information is fresh in the counsellor's mind.

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NHS IAPT SERVICES

NHS 75 England

Home > Mental health > Adult and older adult mental health > Adult Improving Access to Psychological Therapies programme

Adult Improving Access to Psychological Therapies programme

The Improving Access to Psychological Therapies (IAPT) programme began in 2008 and has transformed the treatment of adult anxiety disorders and depression in England. IAPT is widely recognised as the most ambitious programme of talking therapies in the world and in the past year alone more than one million people accessed IAPT services for help to overcome their depression and anxiety, and better manage their mental health.

Plans set out in the NHS Long Term Plan build on the ambitions of the [5 Year Forward View for Mental Health](#), and will see the number of people with anxiety disorders or depression who can access talking therapies through IAPT increase by an additional 380,000 per year to reach 1.8 million by 2023/24. Details of local IAPT services are available on the [IAPT website](#). The [NHS Mental Health Transformation Plan 2023/24 - 2027/28](#) provides a new framework to ensure delivery, at the local level, on the commitment to pursue the most ambitious transformation of mental health care in England.

IAPT services are characterised by three things:

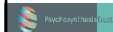
- 1. Evidence-based psychological therapies:** with the therapy delivered by fully trained and accredited practitioners, matched to the mental health problem and its intensity and duration, delivered as evidence-informed. From April 2028, all eligible commissioning services are expected to offer:

Psychology Training

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ICD-10 VS DSM-5

- The *Diagnostic and Statistical Manual of Mental Disorders, Fifth Edition* (2013). First published in 1952 by the American Psychiatric Association. In the USA, the DSM serves as the principal authority for psychiatric diagnoses. Treatment recommendations and payments by health care providers, are often determined by DSM classifications.
- DSM Criticisms:
 - 1) many DSM-5 revisions or additions lack empirical support;
 - 2) inter-rater reliability is low for many disorders;
 - 3) several sections contain poorly written, confusing, or contradictory information;
 - 4) the psychiatric drug industry has influenced the manual's content;
 - 5) many DSM-5 workgroup participants had ties to pharmaceutical companies.



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ICD-10 VS DSM-5 – CONT

- ICD stands for the International Statistical Classification of Diseases and Related Health Problems, or "International Classification of Diseases" for short.
- ICD is published by the World Health Organisation (WHO), the ICD sets the international standard for healthcare diagnosis.
- WHO is a specialised agency of the United Nations which monitors and supports public health in around 180 countries.
- ICD-10 is looks at conditions from a global perspective, and is a trusted source in many countries.
- There is argument that the ICD-10 is a better reference than the popular DSM-V.



122

DSM TABLE OF CONTENTS – PART 1

- • Neurodevelopmental Disorders
- • Schizophrenia Spectrum and Other Psychotic Disorders • Bipolar and Related Disorders
- • Depressive Disorders
- • Anxiety Disorders
- • Obsessive-Compulsive and Related Disorders
- • Trauma- and Stressor-Related Disorders
- • Dissociative Disorders
- • Somatic Symptom Disorders
- • Feeding and Eating Disorders
- • Elimination Disorders



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DSM TABLE OF CONTENTS – PART 2

- • Sleep-Wake Disorders
- • Sexual Dysfunctions
- • Gender Dysphoria
- • Disruptive, Impulse Control and Conduct Disorders
- • Substance Use and Addictive Disorders
- • Neurocognitive Disorders
- • Personality Disorders
- • Paraphilic Disorders
- • Other Disorders



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QUESTION:
MENTAL HEALTH LABELS – PROS AND CONS?

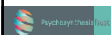
- What are the 'CONS' of labels?
- What – if any – are the 'PROS' of labels?



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CORE-10 / CORE-34

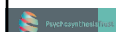
- CORE stands for "Clinical Outcomes in Routine Evaluation". CORE is used to show the progress of therapy and has a range of Clinical Outcome Measures.
- There is a short version (CORE-10) and a longer version CORE-34
- CORE can be administered in a variety of ways. At a minimum, it is administered in the first and final sessions. The IAPT services uses these for each session.



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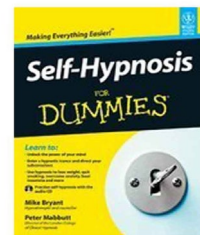
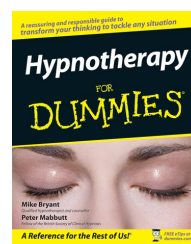
HYPNOSIS VIDEOS / RESOURCES

- Videos: Intro to Self Hypnosis Part One: Theory
- <https://youtu.be/4HpefxYmVyg>
- Intro to Self Hypnosis Part Two: Self-Hypnosis Demo
- https://youtu.be/lFY8QI_ONx0



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HYPNOSIS BOOKS BY MIKE BRYANT



128

EATING DISORDERS

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DSM-5 E.D. CATEGORIES

Feeding and Eating Disorders

3.10.1 Differential Diagnosis for Avoidant/Restrictive Food Intake Disorder	244
3.10.2 Differential Diagnosis for Anorexia Nervosa	246
3.10.3 Differential Diagnosis for Bulimia Nervosa	249
3.10.4 Differential Diagnosis for Binge-Eating Disorder	251

130

EATING DISORDER OR DISORDERED EATING ?

- **Eating disorders are clinical diagnoses.** This means that they meet the criteria for a specific eating disorder, such as anorexia nervosa, bulimia nervosa, or binge eating disorder. These disorders are characterized by a persistent disturbance in eating or weight-related behavior that significantly impairs physical or psychological functioning.
- **Disordered eating is not a clinical diagnosis.** It is a term used to describe a range of unhealthy eating behaviors that do not meet the criteria for a specific eating disorder. These behaviors can include restrictive dieting, binge eating, purging, and excessive exercise.
- **Disordered eating can also be a precursor to an eating disorder.** The severity of disordered eating can vary from person to person. Some people may only experience occasional disordered eating behaviors, while others may engage in these behaviors on a regular basis.



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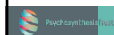
THINKING ABOUT EATING DISORDERS 1/2

- **A client in the grips of Anorexia Nervosa requires additional mental health support as this can result in severe physical and emotional harm or even death.**
- Is it an eating disorder or disordered eating ?
- Explore cultural beliefs? Comparisons to other races and cultures?
- Internalised discrimination and primal wounding of not being accepted for looking a certain way? Being different and not belonging.
- Is this about food or emotional issues? Is this about health?
- Or is this to do with an internalised inner critic? (parental judgements?)

132

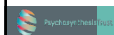
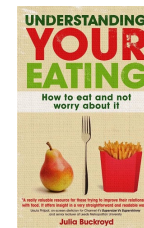
THINKING ABOUT EATING DISORDERS 2/2

- Has there been an overemphasis on the clients weight by family members, partners? Fat shaming.
- Is this about bullying and or low self esteem?
- Is this about body image or perception of the clients own body?
- Is weight being used as a protection from intimacy ?
- Is it an act of rebellion?
- Is this about not being a child anymore and becoming a woman or man and gender identity?
- What else?



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BOOK: 'UNDERSTANDING YOUR EATING' BY JULIA BUCKROYD



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THINKING ABOUT EATING DISORDERS

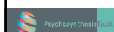
- **A client in the grips of Anorexia Nervosa requires additional mental health support as this can result in severe physical and emotional harm or even death**
- Is it an eating disorder or disordered eating ?
- Explore cultural beliefs? Comparisons to other races and cultures?
- Internalised discrimination and primal wounding of not being accepted for looking a certain way? Being different and not belonging.
- Is this about food or emotional issues? Is this about health?



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THINKING ABOUT EATING DISORDERS

- Or is this to do with an internalised inner critic? (parental judgements?)
- Has there been an overemphasis on the clients weight by family members, partners? Fat shaming.
- Is this about bullying and or low self esteem?
- Is this about body image or perception of the clients own body?
- Is weight being used as a protection from intimacy ?
- Is it an act of rebellion?
- Is this about not being a child anymore and becoming a woman or man and gender identity?
- What else?



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ANOREXIA NERVOSA

SYMPTOMS:

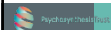
- People w anorexia try to keep their weight as low as possible by not eating enough or over-exercising, or a combination of the two.
- Thinking you are overweight even if others say you are dangerously thin
- Low self-esteem or negative self-image
- Fear of gaining weight
- Dismissive of the idea of eating more or encouragement from others to do so



137

BULIMIA NERVOSA

- Fear of gaining weight
- Low self-esteem or negative self-image
- Experiencing mood changes such as anxiety or tension



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BULIMIA NERVOSA

Expressed via

- 'Binging': over-eating to numb pain.
- 'Purging': vomiting / defecating via laxatives



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WORKING WITH EATING DISORDERS? 1/2

- Clarify the issues
- Work towards client acceptance of body shape
- Work with client health in mind and not client body shape
- Do not comment on changes ie loss of weight, putting on weight, attractiveness etc. Client will try and please you or rebel against you if they get angry and or play out early patterns with family.



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WORKING WITH EATING DISORDERS? 2/2

- Do not give dietary advice refer them to the internet and NHS websites for advice on healthy eating.
- Be aware of intersectional issues in the room eg are you thin, slim or heavy? How does this play out in the transference.
- Work with inner child and adverse consequences of bullying, abuse, shame etc.
- Explore underlying issues rather than focusing on what they currently eat.
- Monitor bulimia if present and link to psychological/emotional issues stressors

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EXERCISE: EATING DISORDER CASE STUDIES

- Two groups
- Two separate Case Studies for Eating Disorder Cases.
 - Case 1 – 'KIM'
 - Case 2 – 'HANNAH'
- PRACTICUM
 - - 20 MINUTES TO DISCUSS AND DEVEISE a TREATMENT PLAN

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On line Assessment & Person to Person Assessment

On line

- Can see client in context of their environment
- May disclose more quickly
- Easier for client to 'stage manage' presentation of survival personality
- Client may be more relaxed as they are in their natural environment or more stressed depending on home environment – may feel less safe and less confidential if other people at home
- Other factors...

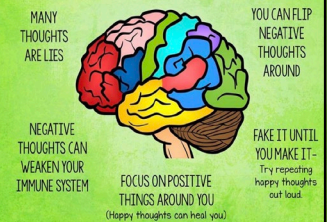
Person to person

- You can see more of the client
- May notice something that can be kept hidden on line
- Less interruptions than zoom
- Clearer picture
- Can be a safe neutral space
- Can detect body odour, perfume etc
- Can adjust physical proximity
- Other factors...

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NEGATIVE AUTOMATIC THOUGHTS

**Don't believe every
thought you have.**



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NEGATIVE AUTOMATIC THOUGHTS

Can you combine an initial assessment with therapy?

- see 'Negative Automatic Thoughts' handout -

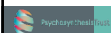


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EXERCISE: EXPLORING NEGATIVE THOUGHT PATTERNS AS AN ASSESSMENT TOOL

- Ask client to give you an example of a dilemma they are facing OR an area that they feel stuck with that they would like to explore in therapy.
- Give them all the time they need to scan the list of negative thoughts
- Ask them to pick one or a maximum of two that they experience related to the problem and explore
- Use a Scale of your Choice – CORE-10, GAD-7, PHQ-9 in any way – wholly or selected questions - into your conversation / assessment.

TIME: 20 minutes each in pairs.
TOTAL TIME: 40 minutes



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Reading List

- Gelinas, C.J. & Hayes, J. (2007) *Countertransference & the Therapist's Inner Experience: Paths and Possibilities*. New Jersey: Lawrence Erlbaum Associates, Inc.
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- Moore, T. (1994) *Care of the Soul*. New York: HarperPerennial.
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- Casement, P.J. (1991) *On Learning from the Patient*. New York: Penguin Press.
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- Gomez, L. (1997) *An Introduction to Object Relations*. Free Association Books.
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